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M.Ch. (Urology) Practice Paper – Set 9 – Paper I
BASIC SCIENCES IN UROLOGY – I (compiled from Question Bank)

Total Duration : 3 Hours Total Marks : 100

Instructions: 1) All questions are compulsory. 2) The number to the right indicates full marks. 3) Draw diagrams wherever necessary. 4) Questions can be asked from any paper's syllabus into any question paper. 5) Use a common answer book for all sections.

1. Long Answer Question : [1 × 25 = 25]

Describe in detail, anatomy of prostate with special reference to concepts in surgical management if localised CA prostate.

2. Long Answer Question : [1 × 25 = 25]

Describe anatomy of pelvic floor in females. Discuss fertility preserving treatment options for procidentia

3. Short Answer Questions (Solve any 5 out of 6) : [5 × 10 = 50]

a) Describe contribution of a. Prof. Herald Hopkins b. Dr Charles Huggins c. Prof. Roy Caine d. Dr D D Gaue
b) EMG
c) Secondary malignancies in Uro
d) Hematospermia
e) Botulimum toxin
f) Retrograde endopyelotomy

Source of each question: *Q1: Bank (LAQ, OE) | Q2: Bank (LAQ JUN 12) | 3a: Bank (no tag printed) | 3b: Bank (SN OE) | 3c: Bank (SN JUN 12) | 3d: Bank (SN May 10, SN OE) | 3e: Bank (SN JUN 10 OE) | 3f: Bank (SN OE)*

[Total No. of Pages : 1] **14012**

M.Ch. (Urology) Practice Paper – Set 9 – Paper II
UROLOGY – II (compiled from Question Bank)

Total Duration : 3 Hours Total Marks : 100

Instructions: 1) All questions are compulsory. 2) The number to the right indicates full marks. 3) Draw diagrams wherever necessary. 4) Questions can be asked from any paper's syllabus into any question paper. 5) Use a common answer book for all sections.

1. Long Answer Question : [1 × 25 = 25]

Diagnosis and Treatment of renal trauma

2. Long Answer Question : [1 × 25 = 25]

Discuss etiopathogenesis of uric acid calculi. Describe rationale of medical management,

3. Short Answer Questions (Solve any 5 out of 6) : [5 × 10 = 50]

a) Methods for GFR estimation in prospective tx donor.
b) Measures to decrease complications of TURP
c) What is optical coherence tomography. What is its potential role in management of CA bladder?
d) Tubeless PCNL
e) Medullary sponge kidney
f) Amendments to THOA: 2011.

Source of each question: *Q1: Bank (LAQ OE) | Q2: Bank (LAQ OE) | 3a: Bank handwritten (SAQ '16) | 3b: Bank () | 3c: Bank (SN JUN 14) | 3d: Bank (SN OE) | 3e: Bank (SN MAY 09 OE); Bank (SN OE) | 3f: Bank handwritten (SAQ undated)*